co-op.care Master Outreach Pipeline

All Four Audiences — Targets, Priority, Messaging, Draft Outreach

Last Updated: March 7, 2026 **Infographic Assets:** FOR-INVESTORS.jsx, FOR-PHYSICIANS.jsx, FOR-PARTNERS.jsx, FOR-FAMILIES.jsx

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SEGMENT 1: INVESTORS — COOPERATIVE CAPITAL STACK

The right capital stack for a worker cooperative is NOT venture capital. These are mission-aligned lenders, cooperative-specific funders, impact investors, and foundations with explicit workforce and aging mandates. The pitch: community-owned infrastructure, not a startup to flip.

Tier 1 — Purpose-Built Cooperative Funders (Approach First)

RSF Social Finance

Why first: The premier mission-aligned lender for cooperatives. Has funded home care cooperatives directly. They understand Subchapter T, patronage dividends, and cooperative governance — you don't have to explain the model. **Ask:** Working capital loan or catalytic capital for pre-revenue cooperative launch. **Attach:** FOR-INVESTORS infographic + investor one-pager. **Contact path:** Direct application through RSF lending program. Mention cooperative home care precedent.

DRAFT EMAIL:

Subject: Worker-owned home care cooperative seeking catalytic capital — Boulder, CO

We're launching co-op.care, a worker-owned home care cooperative in Boulder, Colorado. We're pre-revenue (LLC filed March 3, 2026) with two physician co-founders, a five-layer clinical safety architecture backed by Lloyd's of London and Swiss Re, and a blended care model combining professional caregivers ($25-28/hr W-2) with a community Time Bank.

RSF's track record funding home care cooperatives directly is why we're reaching out to you first. The cooperative structure isn't a feature of our model — it IS the model. Worker ownership eliminates private equity extraction, reduces turnover from 77% to under 15%, and creates the community embeddedness that no purely technology company can replicate.

The timing: ACA premiums just surged 26% nationally (101% in Colorado). The federal government is depositing cash directly into consumer Health Savings Accounts. Our physician's oversight makes home care eligible for those accounts. We're the vehicle that catches those dollars.

Year 3 core revenue projection: $2.6 million (high confidence commercial layers only). Full scenario with federal programs and cooperative insurance: $4.1 million.

I'd welcome 20 minutes to walk through the model. Investor overview attached.

Blaine Warkentine, MD
blaine@co-op.care | 484-684-5287

National Cooperative Bank

Why: Lends specifically to worker cooperatives including home care. Understands cooperative capital structures, patronage dividends, and member equity. **Ask:** Cooperative business loan or line of credit for operational launch. **Attach:** FOR-INVESTORS infographic + investor one-pager.

DRAFT EMAIL:

Subject: Home care worker cooperative — Boulder, CO — seeking cooperative lending

co-op.care is a worker-owned home care cooperative launching in Boulder, Colorado. We're structured as a cooperative from day one — caregivers earn $25-28/hr as W-2 employees and build $52,000 in ownership equity over five years through patronage dividends under Subchapter T.

We're seeking cooperative lending to bridge the period between company formation (March 2026) and state home care licensure (projected mid-summer 2026). Revenue begins with physician-supervised clinical assessments ($150-300, Health Savings Account eligible) in May, scaling to a 10-layer revenue stack generating $2.6 million in core revenue by Year 3.

Our model blends professional caregivers with a structured community Time Bank — cutting family costs 44% while paying workers nearly double the industry median wage. $25 million in malpractice and technology insurance through Lloyd's of London. AI performance warranty through Swiss Re.

Would appreciate 20 minutes to discuss cooperative lending options.

Blaine Warkentine, MD
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Cooperative Fund of New England (CFNE)

Why: Has funded Colorado cooperatives via the Rocky Mountain Employee Ownership Center pipeline. Understands the RMEOC network and Colorado cooperative ecosystem. **Ask:** Cooperative development loan. Mention RMEOC connection explicitly. **Attach:** FOR-INVESTORS infographic.

DRAFT EMAIL:

Subject: Colorado worker cooperative — home care — via RMEOC pipeline

We're launching co-op.care, a worker-owned home care cooperative in Boulder, Colorado. Writing because CFNE has funded Colorado cooperatives through the Rocky Mountain Employee Ownership Center pipeline, and we're actively engaging with RMEOC on our cooperative formation.

Quick overview: professional caregivers earning $25-28/hr as W-2 employee-owners, blended with a community Time Bank that cuts family costs 44%. Two physician co-founders. $25 million in insurance through Lloyd's of London. AI-powered documentation platform with mandatory human review on every clinical note.

Seeking cooperative development capital for our pre-revenue launch phase. Revenue begins May 2026 with clinical assessments. Year 3 core projection: $2.6 million.

20 minutes to discuss?

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Colorado OEDIT Employee Ownership Office

Why: State grants and loans specifically for cooperative formation. This is free or near-free money earmarked for exactly what you're building. **Ask:** Cooperative formation grant/loan. Technical assistance. **Approach:** Application-based. Straightforward — you're a Colorado cooperative in formation.

DRAFT EMAIL:

Subject: Worker-owned home care cooperative seeking formation support — Boulder

co-op.care is a worker-owned home care cooperative forming in Boulder, Colorado (LLC filed March 3, 2026). We're seeking cooperative formation grants and technical assistance through the Employee Ownership Office.

Our model: professional caregivers earning $25-28/hr as W-2 worker-owners with equity accrual, combined with a community Time Bank that reduces family costs 44%. Two physician co-founders. Targeting 200 founding families and 10 initial worker-owners in 2026, scaling to 100+ worker-owners by 2027.

We fit squarely within OEDIT's employee ownership mandate — this is a cooperative addressing Colorado's caregiver workforce crisis (third-fastest aging population nationally, 8,700 caregiver shortfall) by creating AI-proof, wealth-building careers in a sector with 77% annual turnover.

What's the best path to apply?

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Tier 2 — Colorado Impact Investors and Foundations

Colorado Health Foundation

Why: Grants plus impact investment with explicit workforce and aging focus. This is their mandate. **Ask:** Impact investment or program-related investment. Possibly grant for community health infrastructure. **Hook:** Colorado premium crisis + cooperative workforce model + aging population.

DRAFT EMAIL:

Subject: Worker-owned home care cooperative addressing CO workforce and aging crisis

225,000 Coloradans just saw health insurance premiums double. Colorado has the third-fastest aging population in the nation and an 8,700 caregiver shortfall. 77% of home care workers quit every year because private equity extraction suppresses their wages to $16.78/hr.

co-op.care is a worker-owned cooperative that solves both sides: families get affordable care (44% less than traditional agencies, Health Savings Account eligible), and caregivers get $25-28/hr W-2 employment with health coverage and $52,000 in ownership equity over five years.

This sits at the intersection of Colorado Health Foundation's workforce and aging mandates. We're pre-revenue (formed March 2026), targeting 200 founding families in Boulder County, with two physician co-founders and $25 million in insurance through Lloyd's of London.

Seeking either impact investment for cooperative launch or grant funding for community health infrastructure development. Would welcome a conversation about how this fits within your current programs.

Blaine Warkentine, MD
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Gary Community Ventures (Denver)

Why: Major Colorado impact investor. Health and economic mobility focus. Denver-based, understands the Colorado landscape. **Ask:** Impact investment or accelerator-style engagement. **Hook:** Economic mobility via worker ownership + health access via cooperative care.

DRAFT EMAIL:

Subject: Boulder worker cooperative — health access + economic mobility

co-op.care is a worker-owned home care cooperative turning the caregiving crisis into an economic mobility engine. Caregivers earn $25-28/hr (nearly double industry median) as full employees with health insurance and build $52,000 in ownership equity over five years.

For families: 44% cost reduction versus traditional agencies. Health Savings Account eligible. In a market where 225,000 Coloradans just saw premiums double, we're the affordable alternative to $9,900/month nursing homes.

Two physician co-founders, three prior exits, $25 million insurance through Lloyd's of London. Pre-revenue, forming in Boulder, targeting 200 founding families for 2026 launch.

This is health access and economic mobility in one model. Would appreciate 20 minutes to walk through it.

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Unreasonable Group (Boulder)

Why: Impact accelerator in your backyard. They understand Boulder. Application-based but the personal proximity matters. **Ask:** Accelerator cohort inclusion. Mentorship + network access. **Hook:** You're local. The model is globally relevant (China/Switzerland national time banking). The cooperative structure is the impact thesis.

DRAFT EMAIL:

Subject: Worker-owned home care cooperative — Boulder-based, globally relevant model

I'm three miles from your office building a worker-owned home care cooperative. co-op.care combines professional caregivers ($25-28/hr W-2 with equity) with a community Time Bank — the same model China and Switzerland are building into national elder care policy. We're doing it at neighborhood scale in Boulder.

Three prior exits in healthcare technology. Two physician co-founders. $25 million insurance through Lloyd's. AI performance warranty through Swiss Re. Pre-revenue, launching 2026.

The unreasonable part: we think community ownership beats private equity extraction in a $100 billion market where 77% of workers quit every year. The cooperative structure isn't a philosophy — it's a retention moat and an actuarial advantage for the insurance product we're launching in 2027.

Is there a current or upcoming cohort where this fits?

Blaine Warkentine, MD
blaine@co-op.care | 484-684-5287

Tier 3 — National Foundations and Strategic Angels

Brad Feld / Foundry Group

Why: Anchor Boulder tech philanthropist. In your Tier 3 investor list. Understands Boulder, cooperatives, and impact. His public writing on community building aligns. **Ask:** Angel check or intro to his foundation/giving network. Not a Foundry GP deal — a personal allocation. **Approach:** Warm intro preferred. Check your 57K contacts for mutual connections.

Steven Krein / StartUp Health

Why: Health innovation fund. If already in your pipeline, accelerate. If not, the "10-layer revenue stack from one visit" is exactly the health innovation narrative they fund. **Ask:** Health innovation fund investment. Moonshot Academy membership.

Kresge Foundation

Why: Community health and economic mobility. Has funded home care cooperatives nationally. Major institutional funder. **Ask:** Program-related investment or grant. Their Kresge Health program explicitly funds community health infrastructure. **Hook:** Worker-owned cooperative + community Time Bank + HSA tax advantage for underserved families.

Robert Wood Johnson Foundation

Why: Workforce and aging focus. Specifically funded the Paraprofessional Healthcare Institute's home care cooperative research. They know this model works — you're implementing it. **Ask:** Grant for cooperative care model implementation and evaluation. **Hook:** "You funded the research at PHI. We're building the implementation."

Investor Outreach Prioritization

Priority	Target	Ask	Timeline
1	Colorado OEDIT Employee Ownership	Grant/loan application	This week
2	RSF Social Finance	Catalytic capital	This week
3	National Cooperative Bank	Cooperative lending	This week
4	Colorado Health Foundation	Impact investment/grant	Week 2
5	CFNE (via RMEOC)	Development loan	Week 2
6	Gary Community Ventures	Impact investment	Week 2
7	Unreasonable Group	Accelerator application	Week 3
8	Brad Feld	Angel / warm intro	When intro available
9	Kresge Foundation	Program-related investment	Week 3-4
10	Robert Wood Johnson Foundation	Grant	Week 3-4
11	Steven Krein / StartUp Health	Fund investment	Week 4

SEGMENT 2: INSTITUTIONAL PARTNERS

Attach FOR-PARTNERS infographic to all outreach below.

Healthcare Organizations

Boulder Community Health (BCH)

Status: In discussion. Not signed. **Ask:** Signed letter of intent for "Safe Graduation" discharge pilot. **Why urgent:** This is the single institutional anchor that validates everything else. Their 15.4% 30-day readmission rate is the cost justification. One signed LOI changes the founding family recruitment trajectory. **Contact:** Hospital administration + discharge planning leadership.

Longmont United Hospital (Centura/CommonSpirit)

Why: Second major health system in your service territory. Expands geographic coverage. CommonSpirit has system-level interest in community health. **Ask:** Parallel discharge pilot discussion. If BCH signs first, Longmont follows. **Hook:** "Boulder Community Health is exploring a discharge pilot with us. We'd like to discuss the same model for your service area."

CU School of Public Health

Why: Academic validation + grant co-application potential. Aging and workforce research credibility. **Ask:** Research partnership for cooperative care model evaluation. Co-applicant on MAHA ELEVATE or similar federal grants. **Hook:** "We're implementing the cooperative home care model that PHI researched. We need an academic partner to evaluate outcomes."

Colorado Gerontological Society

Why: Direct connection to geriatric care professionals statewide. **Ask:** Speaking opportunity + professional network referrals. **Hook:** "Worker-owned cooperative model with AI-powered documentation and community Time Bank — would your members want to hear about this?"

Employers

Boulder Valley School District (BVSD)

Status: In discussion. Not signed. **Ask:** Signed letter of intent for employee caregiver support benefit. **Numbers:** 6,000+ employees. $3-6 per employee per month. Net savings $5,365 per caregiver-employee per year in reduced absenteeism.

City of Boulder HR Director

Why: Large employer, same benefit angle as BVSD. Municipal credibility. **Ask:** Employee benefit pilot. **Hook:** "BVSD is exploring caregiver support as an employee benefit. Your employees face the same pressures. Here's what it looks like for a municipal employer."

CU Employee Benefits

Why: 4,000+ employees in Boulder. Many in the Alpha Daughter demographic. **Ask:** Employee benefit program or founding family channel. **Hook:** Direct to HR/benefits team. "Your employees who are caring for

aging parents miss 6.6 more days per year."

Naropa University (Boulder)

Why: Values-aligned employer. Contemplative care training angle. Small but high-signal partnership. **Ask:** Employee benefit + potential contemplative care curriculum collaboration. **Hook:** "A worker-owned cooperative grounded in community reciprocity and dignified aging. This is Naropa's mission applied to home care."

Community Organizations

Boulder Jewish Community Center

Why: Senior programming. Directly serves the Alpha Daughter family demographic. **Ask:** Community partnership. Founding family referral channel. Time Bank anchor. **Hook:** "Your senior programs serve the exact families we're built for. Partnership = your members get priority access."

Jewish Family Service of Colorado

Why: Aging services and caregiver support programs statewide. **Ask:** Referral partnership. Potential co-programming. **Hook:** "Your caregiver support clients are our founding families."

Erie/Lafayette Senior Centers

Why: Community anchor in adjacent communities. Founding family referral pipeline for Phase 2 expansion. **Ask:** Community presentation + referral partnership.

Elevations Credit Union

Status: In discussion. **Ask:** Financial infrastructure partnership — Care Card, escrow, deposit management.

SEGMENT 3: PHYSICIANS AND CLINICAL PROVIDERS

Attach FOR-PHYSICIANS infographic to all outreach below.

CU Geriatrics Division — Dr. Stacy Fischer

Why: Known home and community-based services researcher. Academic validation. Potential Medical Director advisory board member. **Ask:** Advisory board seat. Research collaboration. Grant co-application. **Hook:** "We're implementing a worker-owned cooperative model with AI-powered documentation, mandatory physician oversight, and five-layer clinical safety architecture. We need academic geriatric expertise on our advisory board."

Boulder Medical Center Primary Care Physicians

Why: Referral source. Founding family channel. These are the doctors whose patients' families are asking "what do I do about Mom?" **Ask:** Referral partnership. Clinical assessment referral flow. **Hook:** "When your patients' families ask about home care options, we're the physician-overseen, cooperative alternative to the agencies with 77% turnover."

Geriatric Care Managers (National Academy of Certified Care Managers — Colorado chapter)

Why: Professional referral network. These are exactly the people who send families to home care agencies. **Ask:** Professional referral relationship. Speaking opportunity at chapter meeting. **Hook:** "We pay caregivers $25-28/hr, target under 15% turnover, and have a physician reviewing every care plan. Your clients deserve better than the agencies you're currently forced to recommend."

Boulder Community Health Discharge Planners

Why: Post-acute transition referral. These are the people who decide where patients go after discharge. **Ask:** Referral workflow integration. "Safe Graduation" pilot participation. **Hook:** "Every readmission costs your hospital $15,000+ and triggers federal penalties. We put a caregiver in the home within 24 hours of discharge."

Visiting Angel / Home Instead Franchise Owners (Boulder area)

Why: Potential acquisition or conversion targets. The International Cooperative Alliance has a cooperative conversion framework. **Ask:** Exploratory conversation about cooperative conversion. **Hook:** "You built a good business serving families. What if your caregivers owned it, earned double the wages, and you got to exit with your legacy intact?"

Colorado Society of Geriatrics

Why: Speaker opportunity and peer network. **Ask:** Conference presentation slot. **Hook:** "Worker-owned cooperative home care with AI documentation and five-layer clinical safety — a new model for geriatric care delivery."

SEGMENT 4: FOUNDING FAMILIES (Alpha Daughters)

Attach FOR-FAMILIES infographic to all outreach below.
This is marketing and community organizing, not cold email. These are the channels that reach the Alpha Daughter at scale.

Warm Referral Channels

Estate Planning Attorneys (Boulder)

Why: In the room when families have THE conversation about aging parents. **Ask:** Referral partnership. "When you're helping a family plan for aging care, here's the option that's physician-overseen and 44% less expensive." **Leave behind:** FOR-FAMILIES infographic printed or PDF.

Financial Planners — BSW Wealth, Crestone Capital

Why: Same room, same conversation, different angle. These planners are helping families figure out how to pay for care. **Ask:** Referral partnership. The Health Savings Account angle is their language. **Hook:** "Your clients are paying for home care with after-tax dollars. Our physician's Letter of Medical Necessity makes it Health Savings Account eligible — that's 28-36% savings before the cooperative pricing even kicks in."

Elder Law Attorneys (Boulder)

Why: Direct referral from legal planning to care planning. **Ask:** Referral relationship. "When your clients need home care, here's the cooperative option."

Organic Community Channels

Boulder Moms / Local NextDoor Admins

Why: Organic word of mouth in the exact communities you're targeting. **Approach:** Not corporate outreach. Community storytelling. "We're building something in Boulder and looking for 200 founding families."

Local Clergy / Faith Community Leaders

Why: Trusted voices for aging conversations. Many congregations have active senior ministries. **Ask:** Community presentation. "We'd love to talk to your congregation about a new option for aging at home in Boulder."

Employer-to-Employee Channels

CU Employee Benefits

Why: 4,000+ employees, many in the Alpha Daughter demographic. **Approach:** Through benefits team (see Partners segment above). Employee wellness newsletter feature.

Boulder Valley Women's Health

Why: OB/GYN practice reaching the exact Alpha Daughter demographic — women 40-55. **Ask:** Waiting room materials. Referral partnership. **Hook:** "Your patients are the sandwich generation — caring for aging parents while raising kids. Here's the resource."

Community Organizations (Dual-Purpose — Partners + Family Channel)

Boulder JCC Senior Programs → founding family referrals

Jewish Family Service → caregiver support client referrals

Erie/Lafayette Senior Centers → community presentations

Naropa community → values-aligned founding families

OUTREACH EXECUTION CALENDAR

Week 1 (Now)

- [ ] Submit Colorado OEDIT Employee Ownership application
- [ ] Send RSF Social Finance email
- [ ] Send National Cooperative Bank email
- [ ] Send BCH letter of intent request (if not already in motion)
- [ ] Send BVSD letter of intent request (if not already in motion)

Week 2

- [ ] Send Colorado Health Foundation email
- [ ] Send CFNE email (mention RMEOC)
- [ ] Send Gary Community Ventures email
- [ ] Contact CU Geriatrics (Dr. Stacy Fischer)
- [ ] Contact Boulder Medical Center primary care

Week 3

- [ ] Submit Unreasonable Group application
- [ ] Send Kresge Foundation email
- [ ] Contact geriatric care managers (Colorado chapter)
- [ ] Schedule community presentations (JCC, senior centers)
- [ ] Contact estate planning attorneys and financial planners

Week 4

- [ ] Send Robert Wood Johnson Foundation email
- [ ] Contact StartUp Health

- [ ] Pursue Brad Feld warm intro
- [ ] Contact Longmont United Hospital
- [ ] Contact City of Boulder HR
- [ ] Begin NextDoor / community organic outreach